

REGIONAL TRAUMA & POST-SURGICAL CARE HOSPITAL

Surgical Pathology & Hospital Infection Prevention

NABH & NABL ACCREDITED

SURGICAL WOUND CULTURE & ANTIMICROBIAL REVIEW

REF: SW-2026-44019

Patient ID / Alias: PT-1035 (Bed 22) **Prescribing Dept:** General Surgery **Collection Date:** 11-Sep-2026 04:00 PM
Age / Gender: 70 Y / Male **Allergy Status:** CHART DISCREPANCY: ER noted Penicillin Anaphylaxis; Ward notes mild rash. **Current Antibiotic:** ORDER CONFLICT: Meropenem 1g IV vs Pip-Taz 400mg IV

SPECIMEN & MICROBIOLOGICAL IDENTIFICATION

Specimen Type: Deep Surgical Wound Swab / Subcutaneous Organism: **Pseudomonas aeruginosa** ($>10^5$ CFU/mL)
Direct Smear: Numerous polymorphs, non-fermenting Gram-negative bacilli. **Culture Growth:** Moderate to heavy growth with pyocyanin production

ANTIMICROBIAL SUSCEPTIBILITY TESTING (CLSI M100 / EUCAST CRITERIA)

ANTIMICROBIAL AGENT	WHO AWARE	MIC (ug/mL)	INTERPRETATION
Cefepime	Watch Tier (Antipseudomona)	≤ 2.0	SUSCEPTIBLE
Meropenem	Watch Tier	≤ 1.0	SUSCEPTIBLE
Ciprofloxacin	Watch Tier	≤ 0.5	SUSCEPTIBLE
Ceftriaxone	Watch Tier	≥ 64	RESISTANT
Amoxicillin-Clavulanate	Access Tier	≥ 32	RESISTANT
Gentamicin	Access Tier	≤ 2.0	SUSCEPTIBLE

BIOCHEMICAL & ORGAN FUNCTION BIOMARKERS

Serum Creatinine	Total Leukocyte (TLC)	Neutrophils %	Wound Status
1.2 mg/dL	14,200 /uL	84%	Purulent drainage noted

ANTIMICROBIAL STEWARDSHIP CLINICAL SUMMARY & ACTION PLAN

TARGETED RECOMMENDATION: Clarify allergy severity; reconcile MAR to Cefepime 2g IV q8h (7 days).
Clinical Rationale: Reconciles conflicting empiric orders. Sputum/wound isolate is Pseudomonas aeruginosa. Cefepime offers targeted antipseudomonal coverage.
Safety & Monitoring: URGENT: Verify with patient family regarding the nature of the penicillin reaction (anaphylaxis vs benign rash) prior to full implementation.
Stewardship & Economic Impact: Resolves multi-document order conflict, prevents adverse drug events, and maintains effective therapy.